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CUSTOMER	: IDAMAN PHARMA MANUFACTURING SDN BHD
DESCRIPTION	: Fepiril Syrup 120mg 5ml
MEASUREMENT	: W:104mm X H:152mm
MATERIAL	: SMP0 050

SHEET SIZE	: NIL	COLOUR	: Black
PROCESS	: NIL		
FINISHING	: FOLDING		
BOARD OUTING	: NIL		

MATERIAL CODE:	Attn : MS. ZAKIAH From : MALA I hereby verify & confirm that this draft is the final copy. Any subsequent changes of the design and content of the end product will not be borne by THUNDER PRINT SDN. BHD.
Lt-031.09	
MOULD NO : -	Customer Signature & Date

4.1.2022

pharmaniaga®

FEPIRIL SYRUP 120MG/5ML

DESCRIPTION

A pinkish red clear syrup with a raspberry flavor.

COMPOSITION

Each 5 ml contains Paracetamol :120mg

Preservatives:

Methyl Paraben :6 mg
Propyl Paraben :1 mg

PHARMACODYNAMICS

Pharmacotherapeutic group: Other Analgesics and Antipyretics; anilides

ATC Code: N02BE01

Paracetamol is a centrally acting analgesic and antipyretic with minimal anti-inflammatory properties.

Analgesic

The mechanism of analgesic action has not been fully determined. Paracetamol may act predominantly by inhibiting prostaglandin synthesis in the central nervous system (CNS) especially cyclooxygenase (COX)-2 and, to a lesser extent, through a peripheral action by blocking pain impulse generation. The peripheral action may be due to inhibition of prostaglandin synthesis or to inhibition of the synthesis or actions of other substances that sensitize pain receptors to mechanical or chemical stimulation.

Antipyretic

Paracetamol probably produces antipyresis by acting centrally on the hypothalamic heat-regulating center to produce peripheral vasodilation resulting in increased blood flow through the skin, sweating, and heat loss. The central action probably involves inhibition of prostaglandin synthesis in the hypothalamus.

PHARMACOKINETICS

Paracetamol is rapidly and almost completely absorbed from the gastrointestinal tract following oral administration. Approximately 90 to 95% of a dose is metabolized in the liver, primarily by conjugation with glucuronic acid, sulfuric acid and cysteine. An intermediate metabolite, which may accumulate in overdose after the primary metabolic pathways become saturated, is hepatotoxic and possibly nephrotoxic. The half-life of paracetamol, 1 to 4 hours following oral administrations, does not change with renal failure but may be prolonged in acute overdose, in some forms of hepatic disease, in the elderly, and in the neonate. However, it may be somewhat shortened in children. The time for paracetamol to reach its peak concentration is 0.5 to 2 hours and time needed to reach the peak effect is 1 to 3 hours. Duration of action is 3 to 4 hours. It is excreted primarily as conjugates through renal and only 3% of a dose may be excreted unchanged.

INDICATIONS

For relief of fever and mild to moderate pain associated with conditions such as aches due to headaches, colds and flu symptoms, teething pain, toothaches, earache and post vaccination reactions.

RECOMMENDED DOSAGE

For an accurate dosing, weigh your child and work out the dosage to be given based on a 15mg of Paracetamol for every kg body weight.

Eg: If your child weighs 20kg:
20 x 15mg = 300mg = (300mg / 120mg x 5mL) = 12.5mL

Example of dose by weight:

Weight (kg)	Ave. Age(years)	Dose (mg)	Dose (mL)
9-14	1-3	135-210	5.5-9mL
15-20	4-6	225-300	9-12.5mL

These doses may be given every 4 to 6 hours if necessary up to a maximum of 4 doses in 24 hours.

ROUTE OF ADMINISTRATION

Oral

CONTRAINDICATIONS

In patients hypersensitive to paracetamol or any other ingredients/ components of the product.

WARNINGS & PRECAUTIONS

This preparation contains PARACETAMOL.
Do not take any other paracetamol containing medicines at the same time.

Allergy Alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Keep out of reach of children. Consult healthcare provider if patient still needs this medicine two days after vaccination. As with all medicines, if the patient is currently taking any medicine, consult healthcare provider before taking this product. Do not give more than 4 doses in any 24-hour period

Paracetamol provides symptomatic relief only, additional therapy to treat the cause of the pain or fever should be instituted when necessary. Do not exceed recommended dose. Leave at least 4 hours between doses.

Patients should contact their health care provider if symptoms persist (if the pain lasts for more than 10 days, if the redness or fever lasts more than 3 days). Do not give this medicine to your child for more than 3 days without speaking to your doctor or pharmacist.

Paracetamol should be given with care to patients with impaired kidney or liver function. Large doses should be avoided in patients with hepatic impairment. Paracetamol overdose may harm the liver. The hazards of overdose are greater in those with (non-irritic) alcoholic liver disease.

Immediate medical advice should be sought in the event of an overdose, even if the child seems well, because of the risk of delayed serious liver damage.

Patients with rare hereditary problems of fructose intolerance should not take this medicine.

INTERACTIONS WITH OTHER MEDICAMENTS

i) Alcohol, Hepatic Enzyme Inducers or Hepatotoxic Medications.

Chronic alcohol intake can increase hepatotoxicity of paracetamol overdose. Acute alcohol intake may also diminish an individual's ability to metabolize large doses of paracetamol, the plasma half-life of which can be prolonged. The hepatotoxicity of paracetamol, particularly after overdose, may be increased by drugs which induce liver microsomal enzymes such as barbiturates, tricyclic antidepressants and alcohol.

ii) Anticoagulants, Coumarin or Indandione or Indandione Derivative

The anticoagulant effect of warfarin and other coumarins may be enhanced by regular use of paracetamol with increased risk of bleeding. The effect may occur already at daily doses of 2000 mg after 3 days. Occasional doses have no significant effect on bleeding tendency. Increased monitoring of INR values should be done during the duration of the combination and after its discontinuation.

iii) Antivirals.

Regular use of Paracetamol possibly reduces metabolism of Zidovudine (increased risk of neutropenia).

iv) Anticonvulsants and oral contraceptives.

The use of drugs that induce hepatic microsomal enzymes such as anticonvulsants and oral contraceptives may increase the extent of metabolism of paracetamol resulting in reduced plasma concentrations of the drug and a faster elimination rate.

The speed of absorption of paracetamol may be increased by metoclopramide or domperidone and absorption reduced by colestyramine.

PREGNANCY & LACTATION

Pregnancy:

Data in human pregnancy have shown no ill effects due to paracetamol used in the recommended dosage, but patients should follow the advice of their doctor regarding its use. A large amount of data on pregnant women indicate neither malformative, nor feto/neonatal toxicity. Epidemiological studies on neurodevelopment in children exposed to paracetamol in utero show inconclusive results. If clinically needed, paracetamol can be used during pregnancy however it should be used at the lowest effective dose for the shortest possible time and at the lowest possible frequency.

Lactation:

Paracetamol is excreted in breast milk but not in clinically significant quantities. Available published data do not contraindicate breast feeding.

ADVERSE EFFECTS/UNDESIRABLE EFFECTS

Adverse effects of paracetamol are usually mild, though hematological reactions have been reported.

Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

Adverse effects of paracetamol are rare but hypersensitivity including skin rash may occur.

There have been reports of blood dyscrasias including thrombocytopenia and agranulocytosis, but these were not necessarily causality related to paracetamol.

Cases of acute pancreatitis have been reported. Paracetamol has been widely used and reports of adverse reactions are rare, and are generally associated with overdose.

Chronic hepatic necrosis has been reported in a patient who took daily therapeutic doses of paracetamol for about a year and liver damage has been reported after daily ingestion of excessive amounts for shorter periods. A review of a group of patients with chronic active hepatitis failed to reveal differences in the abnormalities of liver function in those who were long-term users of paracetamol nor was the control of the disease improved after paracetamol withdrawal.

Low level transaminase elevations may occur in some patients taking therapeutic doses of paracetamol; these are not accompanied with liver failure and usually resolve with continued therapy or discontinuation of paracetamol.

Nephrotoxic effects are uncommon and have not been reported in association with therapeutic doses, except after prolonged administration.

SYMPTOMS & TREATMENT FOR OVERDOSAGE

Symptoms:

Symptoms of paracetamol over-dosage in the first 24 hours are pallor, nausea, vomiting, anorexia, abdominal pain, hypotension and sweating.

Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe poisoning, hepatic failure may progress to encephalopathy, haemorrhage, hypoglycaemia, cerebral oedema, and death. Acute renal failure with acute tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria may develop even in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported.

Treatment:

Immediate treatment is essential in the management of paracetamol overdose. Despite a lack of significant early symptoms, patients should be referred to hospital urgently for immediate medical attention. Symptoms may be limited to nausea or vomiting and may not reflect the severity of overdose or the risk of organ damage. Management should be in accordance with established treatment guidelines.

Treatment with activated charcoal should be considered if the overdose has been taken within one 1 hour. Plasma paracetamol concentration should be measured at 4 hours or later after ingestion (earlier concentrations are unreliable). Treatment with

N-acetylcysteine may be used up to 24 hours after ingestion of Paracetamol, however, the maximum protective effect is obtained up to 8 hours post-ingestion. The effectiveness of the antidote declines sharply after this time. If required the patient should be given intravenous N-acetylcysteine, in line with the established dosage schedule. If vomiting is not a problem, oral methionine may be a suitable alternative for remote areas, outside hospital. Management of patients who present with serious hepatic dysfunction beyond 24 h from ingestion should be discussed with the NPIS or a liver unit.

EFFECTS ON ABILITY TO DRIVE & USE MACHINES

None.

PACKING SIZES

Plastic Containers of 60ml and 120ml.

STORAGE CONDITIONS

Keep container tightly closed. Store in a dry place below 30°C. Protect from light.

INSTRUCTIONS FOR USE

Shake well before use.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

REGISTRATION NUMBER

MAL19912158XZ

KEEP MEDICINES OUT OF REACH OF CHILDREN

JAUHI UBAT-UBATAN DARI KANAK-KANAK

For further information, please consult your doctor or your pharmacist.

Revision Number : 07
Revision Date : 31 Dec 2021

Lt-031.09 Product Registration Holder and Manufacturer

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